

Testosterone

Patient Education Handout

Men's Health Program · KORB Health Group · For Patient Reference

This handout is for educational reference only. It is not medical advice. Always follow your prescription label, your pharmacy's instructions, and your KORB provider's guidance. If your label differs from anything here, follow your label and contact KORB.

What Is Testosterone Replacement Therapy

Testosterone cypionate is a long-acting injectable form of testosterone. It replaces what your body is no longer producing in adequate amounts, bringing your levels back into a healthy range.

It is a thick, oil-based medication, which is why it comes with a specific type of syringe and two different needles. More on that below.

What It May Help With

- Energy and reduced fatigue
- Muscle mass, strength, and recovery from exercise
- Sex drive and sexual function
- Mood, motivation, and mental clarity
- Body composition, including reduced body fat

Most men notice changes gradually rather than immediately. Energy and mood often shift first, within a few weeks. Changes in body composition and strength typically take three to six months. Your provider will adjust your dose over time based on your labs and how you feel.

How to Give Your Injection

Your medication is a thick oil. It comes with a Luer lock syringe and two separate needles: a wider one to draw the medication out of the vial, and a finer one to inject with. Use both. The oil will not draw properly through the fine needle, and drawing with the injecting needle dulls it and makes the injection hurt more.

Step	What to do
1	Wash your hands. Set out your syringe, both needles, alcohol swabs, and your vial.
2	Wipe the top of the vial with an alcohol swab and let it dry.
3	Attach the wider needle. Draw up the exact amount on your prescription label.
4	Remove that needle and attach the finer needle. Do not draw through the fine needle.
5	Clean your injection site with a fresh alcohol swab and let it dry.
6	Inject, then dispose of the needles and syringe in a sharps container immediately.

Where to inject. If you inject under the skin, use the abdomen, upper thigh, or the fatty area of the buttock. If you inject into the muscle, use the outer thigh, the gluteal muscle, or the shoulder. Your provider will tell you which route you are using and show you the technique.

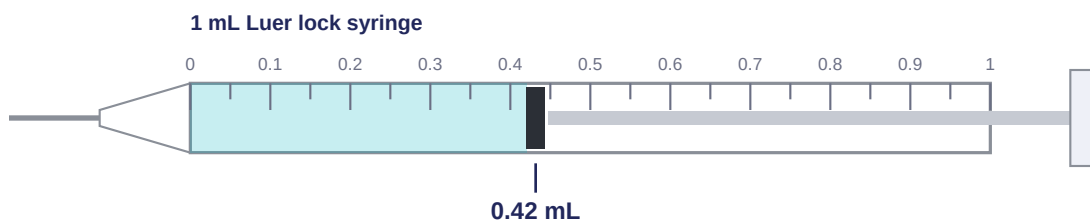
Rotate every time. Move at least an inch or two from your last injection site. Injecting into the same spot repeatedly causes soreness, lumps, and scar tissue that make future injections harder.

Take your time drawing up. The oil moves slowly, and pulling too hard creates air bubbles. Warming the vial briefly in your hand can help it draw more easily.

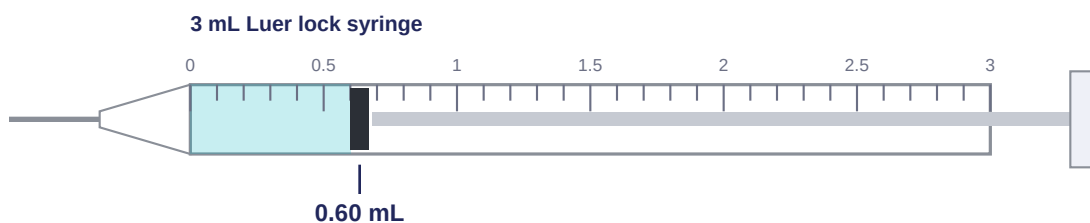
Reading Your Syringe

Your prescription label tells you how many mL to draw. Find that number on the barrel and pull the plunger back until the front edge of the black stopper lines up with it. The examples below show what that looks like.

The numbered lines are tenths of a mL. The smaller unnumbered lines between them are hundredths. So 0.42 mL is four numbered lines past zero, plus two small lines. Draw to the **front edge** of the black stopper, not the middle or back of it.



Example only. Subcutaneous doses use the 1 mL syringe. Your own dose is on your label.



Example only. Intramuscular doses use the 3 mL syringe. Your own dose is on your label.

If you have drawn past your number, do not inject the extra. Push the plunger gently back down until you reach the correct line, or push it all the way in and start again. Injecting more than you were prescribed raises your levels too high and shows up on your labs.

Your Dose

Your prescription label states your exact dose, your volume, and how often to inject. Your provider selects your starting dose based on your labs and symptoms, and may adjust it over time.

Never increase your own dose. More testosterone is not better. Levels that run too high raise your risk of thickened blood, elevated estrogen, and cardiovascular problems, and they do not produce better results. If you feel your dose is not working, tell your provider so it can be adjusted properly and rechecked with labs.

Your Lab Work

Timing matters more than most patients expect. Get your blood drawn on a day you are scheduled to inject, **before** you take that dose. This is called a trough level. If you get drawn right after an injection, your level will read artificially high and your provider may lower a dose that was actually correct.

Labs are ordered about two weeks before your follow-up visit so results are ready when you meet. You will get a reminder when it is time. Your panel checks your testosterone level, your red blood cell count, your prostate marker, and your hormone balance.

Results can take up to a week to come back. That leaves you roughly a one-week window to get drawn once your reminder arrives. Going early is fine. Waiting until the last few days risks your results not being ready, which can delay your visit and your refill.

Refills and Timing

Testosterone is a controlled medication. Pharmacies are legally limited in how early they can release a refill, and they count from the date of your **last fill**, not from the date you run out or the date of your visit.

- Your provider calculates your refill date and sends your prescription a few days before the pharmacy can release it.
- Asking the pharmacy to fill early will not move the date up.
- If you are travelling or think you may run short, tell us well in advance rather than at the last minute.

Discard Your Vial After 90 Days

Throw away your vial 90 days after you first use it, even if medication is left in it. Once a vial has been opened and punctured, it is no longer considered safe to use beyond that point. Write the date you first used it on the label so you know when 90 days is up.

Depending on your dose, there may still be medication in the vial at 90 days. That is expected. Discard it when your new supply arrives rather than stretching it further. Never save leftover medication to use later, and never share it.

Storage and Handling

Store at room temperature	Protect from light	Do not freeze	Keep secured
68°F – 77°F (20°C – 25°C)	Keep in the carton, away from direct sunlight	Do not refrigerate or freeze	Controlled medication. Store out of reach of others.

If the oil looks cloudy, has particles, or has changed color, do not use it. Contact the pharmacy or KORB. Small crystals can form if the vial gets cold; warming it gently in your hand usually clears them. If they do not clear, do not use it.

Side Effects and What to Watch For

What you may notice	What to do
Injection site soreness, redness, or a small lump	Common. Rotate sites and use a fresh needle each time. Tell KORB if it worsens or does not resolve.
Acne or oilier skin	Common, especially early on. Tell your provider if it becomes bothersome.
Fluid retention or mild swelling	Tell your provider. Your dose may need adjusting.
Mood changes, irritability, or trouble sleeping	Tell your provider. This can indicate your dose is too high.
Breast tenderness or swelling	Tell your provider. This may mean estrogen is running high and is adjustable.
Reduced testicle size, or reduced fertility	Expected with testosterone therapy. If you are planning to father children, tell your provider before continuing.

Tell Your Provider Promptly

- Headaches, flushing, or feeling unusually warm, which can suggest thickened blood
- New or worsening snoring, or pauses in breathing during sleep
- Difficulty urinating or a weak stream
- Any new medical diagnosis, new medication, or planned surgery

Seek emergency care immediately for: chest pain, trouble breathing, sudden weakness or numbness on one side, trouble speaking, swelling or pain in one leg, or any symptom that feels severe or unsafe. Do not wait to hear back from KORB.

Safety Reminders

- Use testosterone only as prescribed. Never change your dose, timing, or route without provider direction.
- Use a new syringe and new needles for every injection. Never reuse or share them.
- Draw up with the wider needle, inject with the finer one.
- Do not share this medication with anyone. It is a controlled substance and sharing it is illegal.
- Get your labs drawn at trough, on an injection day before your dose.
- Follow your state and local rules for sharps disposal. Use an FDA-cleared sharps container.
- Tell your provider if you are planning to father children.

When to Contact KORB and When to Seek Emergency Care

Contact KORB Operations	Patient Portal — Message Your Provider	Seek Emergency Care Immediately
• Questions about timing, storage, or administration Email: info@korbhealth.com Phone: +1 (888) 959-7299 Hours: Mon–Fri, 9 AM–6 PM CT	• Mild but persistent side effects • Questions about whether to continue therapy • Questions about injection technique, labs, or refill timing	• Trouble breathing or throat / face swelling • Severe rash or widespread hives • Chest pain or fainting • Severe or rapidly worsening symptoms • Confusion or severe weakness • Any symptom that feels urgent or unsafe

The Patient Portal is your only HIPAA-compliant way to message your provider directly — providers check portal messages once per day on clinic days. Phone and email are not appropriate for emergencies. When in doubt, go to urgent care or the emergency room.

Key Reminders

- Draw up with the wider needle, then switch to the finer needle to inject.
- Rotate your injection site every single time.
- Get labs drawn on an injection day, before your dose.
- Your refill date is set by your last fill date. Early requests will not move it up.
- Discard your vial 90 days after first use, even if medication is left.
- Never increase your own dose. Higher is not better and carries real risk.
- Store at room temperature, not in the refrigerator.